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IFYLLES AV STUDENT OCH TENTAMENSVAKT/
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KURSKOD / COURSE CODE: AK2036	EFTERNAMN / FAMILY NAME: Khodabandehloo
KURSNAMN / COURSE NAME: Vetenskapsteori och vetenskaplig metodik m...	FÖRNAMN / FIRST NAME: Sahand
PROVKOD / TEST CODE: SEM1	NAMNTECKNING / YOUR SIGNATURE:
TENTAMENSDATUM / EXAMINATION DATE (Y/Y/Y/Y - M/M - D/D): 2024 - 12 - 06	PERSONNUMMER / PERSONAL NUMBER (Y/Y/Y/Y/M/M/D/D): 20010921 - 0211

PROGRAMKOD / PROGRAM CODE:	INLÄMNINGSTID / TIME SUBMITTED:	SIGNATUR TENTAMENSVAKT / SIGNATURE INVIGILATOR:	ANTAL BLAD / NO OF SHEETS:
CMEDT	1800	LA	07

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BEDÖMNING / ASSESSMENT

1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20

BONUSPOÄNG/ BONUS POINTS: ,

SLUTSUMMA / FINAL POINTS: ,

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The four basic, universal principles that constitute principlism, a bioethical framework, are autonomy, maleficence, beneficence, and justice. These principles are made practical through specification and balancing judgments, justified by reflective equilibrium.

The first principle, autonomy, refers to an individual's right to decide their course of action regarding their lives, unimpeded by constraints or limits of others. This principle not only encourages choice-making but also protects diverse decisions. Maleficence obliges an actor to non-actively avoid harming others. This is especially important in the case of a doctor treating a patient, e.g., when distress is avoided by not telling about the patient's cancer diagnosis. In contrast, beneficence obliges an actor to actively promote the interests, i.e., in this case, health-related values, conditions, and states, of others. An example is when doctors provide (expected) effective medical treatments of patients.

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Finally, justice, the fourth principle, demands fair and equal treatment and distribution of resources and opportunities to all, i.e., relevant, deserving parts, without biases that may arise due to prejudices, bigotry, or normative descriptions. This principle is, noticably, applied in norms, regulations, and laws.

In relation to the example, following principles are most relevant and applicable:

Autonomy: In light of the freedom of choice, the elderly person has a right to be (fully) informed about the exoskeleton in order to make (informed) decisions and give (informed) consent. Note, the mild (not severe) dementia implies cognitive competence and apprehension of the potentiad risks.

Maleficence: The elderly person might endanger himself/herself or others by going out in a potentially unsafe environment, without help or assistance. Not only would this cause mental distress but this risk is also increased by the cognitive competense loss ...

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... of the dementia (even if it is mild) and age.

Beneficence: Based on promoting well-being, healthcare professionals have an active duty to increase the mobility (physical health) and, thus, improve the elderly person's health-conditions and life-opportunities. Note that mental and social discomforts arise due to the limited (physical) mobility. Furthermore, the prescription of an exoskeleton (directly) saves time and economic efforts, e.g., no need to employ caretakers, which can be put into good use to (indirectly) benefit others.

In conclusion, based on the trade-off between benefits and risk, regarding the cognitively competent person in question, I would argue the principle ranking as follows: first, beneficence, jointly followed by autonomy and maleficence, and, lastly, justice.

As a coherentist (and foundational) theory, principlism inherently faces the same challanges as coherentism, such as the balancing of principles in the (coherent) system, as all principles are...

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... equally morally supporting. In other words, there is no clear, ethical way to compare the basic principles. This problem gets more complicated in the case of the elderly person, where principles are conflicting, as increasing mobility (beneficence) through the use of the exoskeleton also, if no other, limiting action is taken, increases the harm risks (maleficence).

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Paternalism is the act of limiting, restricting, or prevention of another individuals decision, as it is considered to be in that individuals best interest.

The distinction between soft and hard paternalism lies in the informative state of the individuad being paternalized on: soft and hard paternalism is when the paternalized individuad is not and is, respectively, fully informed (of all relevant parameters).

Medical paternalism is the paternalism applied by healthcare professionals or institutions on patients. To be classified as medical paternalism, the following two conditions are jointly sufficient: (1) A healthcare professional or institution restricts a patient's autonomy (2) on the basis that it is in the patients best health-related interest.

The example is a case of medical paternalism as the healtheare professionals, namely, the anesthetists and the surgeon, goes against the will of the patient because they believe it is the best decision for the patient.

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Thus, both criteria of medical paternalism are met.

This example is a good example of conflicting principles in principlism. The four principles are:

- Autonomy - the right of freedom of choice.
- Maleficence - (non-actively) avoid harm.
- Beneficence - (actively) promote well-being.
- Justice - fairness and equal treatment for all.

Applied in the example, I would argue against this practice grounded on the informed decision of the patient (autonomy) and the principle of not directly causing death of another individual (maleficence). First, the patient had, in advance, informed the surgeon about his/her decision to not give consent to get blood transfusions due to her religious beliefs, i.e., violating this decision would go against the patients character and values, which must be respected through the justification of autonomy. Lastly, while the patient did not die in this case, the principle of not directly causing death of others must be taken into account in these practices, as it relates to maleficence.

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Nevertheless, I would also argue to be in favor of this type of practice on the basis of the context of this situation, as well as the duties of the medical team. First, by not doing blood transfusions, there would be a considerable risk the patient would die, thus, rendering the principle of maleficence (avoiding harm) unfulfilled. Second, the blood transfusions were not the initial course of action, but, rather, a consequence of the operation - a risk that the patient might not have been informed on or aware of. At this point, the benefits (patient survives) outweight the risks (patient dies), supported by the principle of doctors must save lives, thus justifying through beneficence. Note, the patient did survive, and, thus, justified further. Lastly, as it differs from blood transfusions, the standard preparative procedure is not violating the patient's autonomy. Rather, since it is a "standard" procedure, it relates more to the principle of justice, ensuring equal treatment.